

العمر(تاريخ الميلاد) : 26-05-1948 المهنة : ربه منزل

المجلس الطبي المتخصص

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Name: حمدية علي محمد ابو خايط National ID: 24805261500409	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

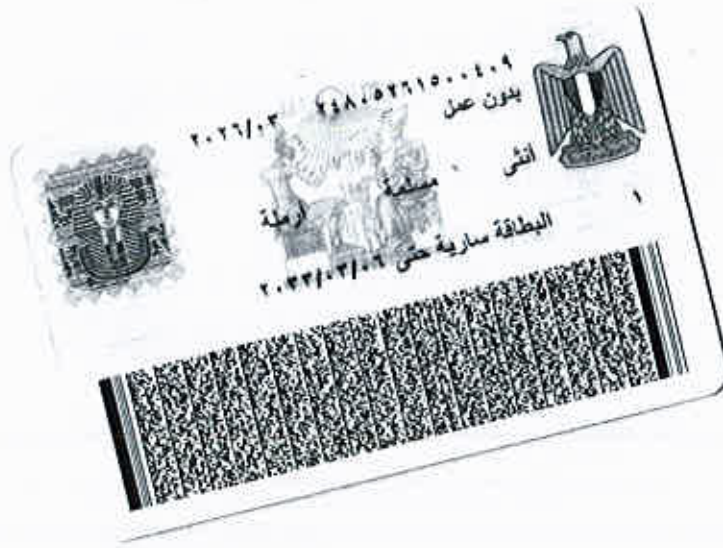
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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A patient of uterine cancer for Further investigations and imaging
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





معامل الخلية

لتحليل الأنسجة والأورام

Patient Name	حمدية على محمد ابو خاطر	Code Number	YK 26004317
Sex/Age	Female 77 Y 8 M 13 D	Patient ID	83832
Registered Date	06/2/2026 02:27:08 PM	Referred By Dr.	محمد جلال
Validation Date	14/2/2026 01:55:35 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	15/3/2026 11:10:38 AM		

Clinical Diagnosis: Postmenopausal Bleeding
Nature of the specimen: D&C Biopsy

Pathology Report

Gross Picture

Multiple grey tan tissue piece collectively measured 3x3x1.5 cm, admixed with blood clots.

Microscopic Picture

Sections examined from endometrial biopsy reveal tissue portions of neoplastic biphasic growth with both epithelial element formed sheets and cords of highly anaplastic epithelial cells and stromal element formed of sheets of high grade, pleomorphic spindle shaped cells with high grade nuclear atypia and variable mitotic activity within focally myxoid stroma.

Diagnosis

Postmenopausal Bleeding, D&C Biopsy:

Mixed Epithelial And Mesenchymal Neoplasm.

Possible Mixed Mullerian Tumor.

For Tight Clinical, Radiological And Tumor Markers Correlation.

The Definitive Final Typing Requires The Excision Biopsy To Provide Adequate Material For Lesion Orientation And Grading.

P.O

Number of slides enclosed: 4

Diagnosed by

Consultant Pathologist

Dr. Hebat Allah Amin, MD



Scan Me



الفرع الرئيسي الدقي : ٢٠ شارع التحرير أمام مترو البحوث - الدقي - الدور الأول
م: ٠١١١٢٤٤٣٣٣ - ٠١١١٥٥٧١١٧ - ت: ٠٢٣٣٣٥٣٠٧٣

Name:	HAMEDYA ALI MOHAMED	Date:	18/02/2026
Referred by:	د/محمد جلال	Age:	78Y

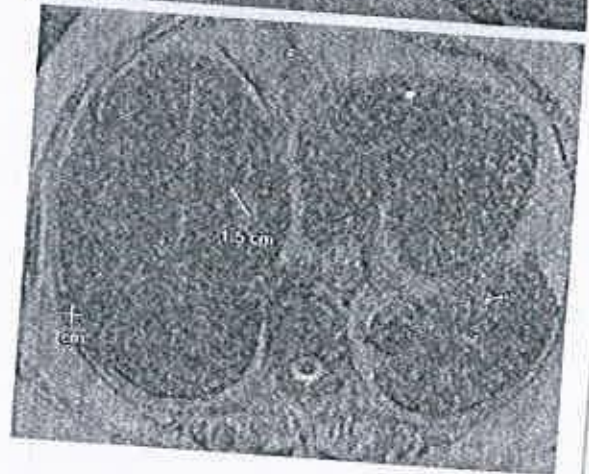
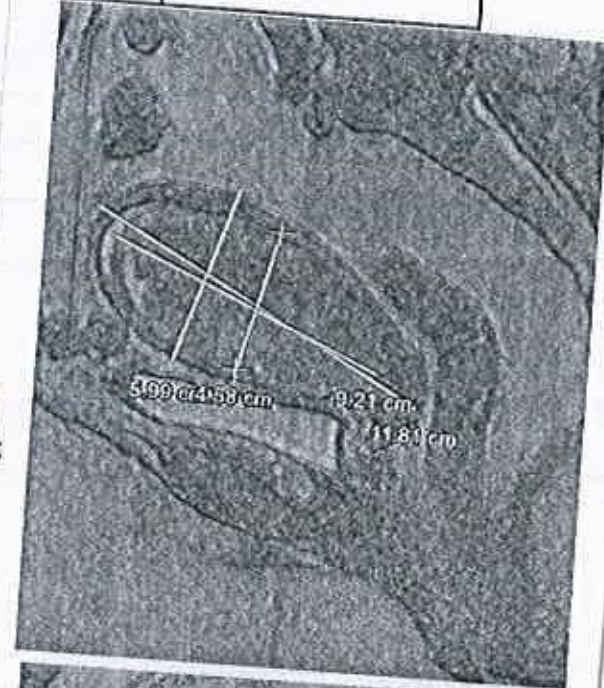
MRI OF THE ABDOMEN PELVIS

MRI TECHNIQUE:

- Axial 4 mm sections with T1 WI and T2 WI
- Coronal 4 mm sections with T1 WI and T2 WI
- Sagittal T2 WI
- DWI and ADC

MRI PELVIS :

- AVF uterus enlarged size (11.7x5.1x 5.9 cm) , distended uterine cavity with heterogeneous mass measures about 9.2x4.5 cm and showing isointense SI in T1 and heterogeneous T2 with area of restricted diffusion showing thinning of the anterior myometrium .
- Normal cervix and endocervical canal
- Small sized both ovaries .
- No adnexal solid or cystic masses .
- Normal MRI appearance of the urinary bladder , rectum and ischiorectal fossae .
- No evidence of enlarged lymph nodes or abnormal fluid collection.



MRI ABDOMEN :

- A small hepatic focal lesion (1.5 cm) is seen in segment 5 , low in T1 , high in T2 . otherwise liver is average in size , no dilated intrahepatic biliary radicles .



- Normal size, shape and signal intensity pattern of the spleen, pancreas and both kidneys.
- No ascites.
- Patent abdominal aorta, IVC and major abdominal & pelvic vascular channels, showing preserved flow void character.
- Two plural bases small lesions seen in both upper lobes 10 mm each

OPINION:

- Endometrial Mass pathologically proven epithelial and mesenchymal neoplasm Stage IB .
- Further metastatic work up is recommended for evaluation of hepatic focal lesion & pleural lesions

Yours truly

Dr. Samar Hosny .MD
Prof . Soha Talaat
Cairo university

فرع الوراق :

٢٧ شارع كورنيش النيل - أمام
فرع التوحيد والفور - ورق الحضر
الوراق.

فرع عزبة الصغايدة :

شارع عبد المنعم عسران - عزبة
الصغايدة - أعلى مسجد أولاد
عمرو - أمبابة.

فرع أمبابة :

شارع نادي أمبابة الرياضي - بجوار
مسجد الجمعية الشرعية - أمام
بنك مصر

ALI MOHAMED, HAMDYA Patient ID: 10110226003781 DOB: 26-May-1948 Date: 24-Mar-2026

Patient Lastname: ALI MOHAMED

Patient Firstname: HAMDYA

HIS Patient ID: 10110226003781

RIS Patient ID: 382563

Date of Birth: 26/05/1948

Procedure: 15722, MRI Pelvis with Contrast

Medical Question:

Clinical Comment: عيادة الجراحة

===== REPORT TEXT =====

Dynamic MRI EXAMINATION OF THE liver

Technique of examination:

" Axial T1 & T2 WIs .

" Coronal T1 & T2 WIs .

" Post contrast axial and coronal T1 WIs.

Comparison : no previous

MRI Findings:

¢ Average size of the liver. No focal hepatic lesions, areas of abnormal enhancement or dilatation of the intra hepatic-biliary radicals.

¢ Normal appearance of the spleen, pancreas, aorta, IVC and both kidneys.

¢ No ascites seen.

¢ No para aortic lymphadenopathy noted.

¢ Normal MRI appearance of the lung bases.

===== REPORT TEXT ENDE =====

Patient Name: حنديه علي محمد ابو خاطر
MRN: 10110226003781
D.O.B: 26/05/1948
IC No.: 24805261500409

Location: الجراحة
Sex: Female
عيادة:

Surgical Pathology-Diagnosis

Slide/Block revision Pathology

Specimen Id 2603240855
Accession No P-26-3484
Sample Type TISSUE

Collection Date 24/03/2026 09:55 AM
Received Date 24/03/2026 02:42 PM
Doctor نساء (أ.ب.د.) جراحة

CLINICAL HISTORY

Endometrial D&C biopsy.

GROSS DESCRIPTION

Two referred paraffin blocks and Four referred Hx & E stained slides coded 26004317.

MICROSCOPIC DESCRIPTION

Section examined revealed a malignant biphasic uterine tumor formed of admixture of malignant epithelial and mesenchymal components. The epithelial component is composed of groups and glands formed of malignant epithelial have vesicular nuclei with prominent nucleoli and pale eosinophilic cytoplasm. The mesenchymal component is formed of sheets of elongated cells have plump spindled to ovoid hyperchromatic and sometimes vesicular nuclei with pale eosinophilic cytoplasm (homologous component). Focal chondroid area is observed (slide labelled 2) There is moderate to marked anaplasia with active mitosis observed in both mesenchymal and epithelial elements.

DIAGNOSIS

Endometrial D&C biopsy: Mixed mullerian tumor.

Category

Malignant primary site (single tumor)

Assistant Pathologist:

Dr. Samar Ahmad

Registrar:

Signed by:

Dr. Shima Zakaria, MD (Lecturer)

Reported and electronically endorsed by Samar Ahmad

Validated: 07/04/2026 06:47 PM

Print Date: 12/04/2026 10:44 AM

Page Number: 1 of 1



Name:	HAMDIA ALI MOHAMMED	Date:	18/02/2026
Referred by:	د / محمد جلال	Age:	78 years

**NON-CONTRAST CT SCAN OF THE CHEST WITH CORONAL
RECONSTRUCTION: -**

Findings:

- Right upper and middle lung lobes mosaic lung attenuation
- Right basal atelectasis
- No CT evidence of enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size and shape. No pericardial effusion detected.
- Upper abdominal cuts are clear.

OPINION:

- Right upper and middle lung lobes mosaic lung attenuation .
- Right basal atelectasis .

Dr. Sheriff Abdel-Fatah, M.D

HALA

فرع الوراق:
٢٧ شارع كورنيش النيل - أمام
فرع التوحيد والنور - ورق الحظير
الوراق.

فرع عزبة الصعايدة:
شارع عبد المنعم عسران - عزبة
الصعايدة - أعلى مسجد أولاد
عمرو - أمبابه.

فرع أمبابه:
شارع نادي أمبابه الرياضي - بصوار
مسجد الجمعة الشرعية - أمام
بنك مصر.



1.1.9

Name	Mrs. Hamdia Ali Mohamed Abo-Khater		
Ref. By Prof. Dr.	Mohamed Ahmed Shama	Date	20/4/2026
ID	1126008546	Acc. No.	291329199

1126008546

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of uterine mixed epithelial and mesenchymal neoplasm (pathology 15.3.2026).

Total body PET scan was done 40 minutes after IV administration of 300 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver SUV... < 2.3

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
(N.B; not all brain metastatic lesions could be detected on ¹⁸-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid (Non-homogenous thyroid gland, for further thyroid work-up) and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- Low activity bilateral axillary lymph nodes, largest about 1.2 cm, likely reactive, for clinical correlation.
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.



Abdomen & Pelvis:

- *Enlarged uterus about 13 X 7 cm eliciting increased metabolic activity of SUVmax. 5.5.*
- *Bulky left adnexa with faint activity eliciting SUVmax. 2.5.*
- Average size of the liver with no FDG hypermetabolic focal lesions. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.

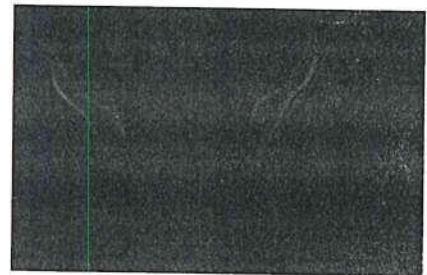


Name	Mrs. Hamdia Ali Mohamed Abo-Khater		
Ref. By Prof. Dr.	Mohamed Ahmed Shama	Date	20/4/2026
ID	1126008546	Acc. No.	291329199

- Average size of the spleen with no focal active lesions. diffuse increased splenic activity of SUVmax. 2.5; likely hematopoietic, for CBC correlation.
- Normal appearance of pancreas (no sizable active masses) and adrenals.
- *Small low activity para-aortic and left iliac lymph nodes, largest about 1 cm of SUVmax. 1.3, however, no enlarged FDG hypermetabolic abdominal or pelvic lymph nodes.*
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- *Left iliac active focus of SUVmax. 3.7.*
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).



Opinion:

- Active uterine enlargement (would fit with the given data) and faint left adnexal activity, for CEMRI pelvis correlation.
- Active osseous focus; warranting follow-up.

Thanks for your referral;

Dr. Ahmed Kandeel
MD Nuclear Medicine

Dr. Islam Aboulwafa, MD
Consultant Radiologist